Nutrition, Sensory, and Neurologic Coordination Assessment

NURSING · nursing-foundations · BoardPrep lesson v1

Official examination scope

PRC Resolution No. 10 s. 2025 and the August 2026 NLE program define examination scope, not topic weights. This lesson teaches a bounded nutrition and sensory assessment foundation from the references below. Open RN and U.S. professional materials support general educational principles only; apply Philippine law, scope of practice, current clinical orders, referral pathways, and facility policy. This study material is not a clinical directive or proof of complete NP coverage. PRC Annex A explicitly places Perception and Coordination within NP IV examination scope; the coordination instruction below is a bounded assessment and change-recognition foundation, not a diagnosis, treatment plan, or local emergency protocol.

Learning objectives

- Collect dietary history, preferences, swallowing, symptoms, intake, hydration, and trends
- before planning nutrition support.
- Respect cultural, religious, and practical food contexts while identifying safety and referral
- needs.
- Distinguish a chronic sensory limitation from a sudden focal change that warrants urgent
- assessment.
- Establish and document a basic neurologic/motor coordination baseline, distinguish a new
- change from baseline, and communicate urgent findings within role.

1. Explain low intake before choosing an intervention

A nutrition assessment asks what the person can and does eat, what has changed, what symptoms interfere, and what is feasible in their life. Review the pattern of intake, swallowing and chewing, nausea or pain, allergies, prescribed diets, hydration, weight trend, food access, preferences, and cultural or religious practices. Confirm chart details with the person when possible. Weakness after several days of very low intake warrants assessment and team communication; it does not justify forcing a large meal or changing a prescribed diet without review. A safe plan is individualized, respects preference, and refers swallowing or clinical concerns through the local pathway.

Sources for this section

Section reference: 14.3 Applying the Nursing Process — Nursing Fundamentals 2e — Open RN; Wisconsin Technical Colleges System — https://wtcs.pressbooks.pub/nursingfundamentals/chapter/14-3-applying-the-nursing-process/
Section reference: Nursing Fundamentals — Open RN; Wisconsin Technical Colleges System — https://wtcs.pressbooks.pub/nursingfundamentals/

2. Separate baseline sensory needs from sudden loss

Ask about usual vision, hearing, sensation, assistive devices, and functional adaptations. A chronic limitation may call for communication and environmental supports, while a sudden focal sensory change—especially with severe headache, weakness, speech change, confusion, or other acute symptoms—requires urgent assessment. Establish onset and progression, assess neurologic and safety changes within role, and use the current emergency pathway. Do not diagnose stroke, reassure as fatigue, or delay because the patient has a prior sensory condition. The cited educational skill resource identifies potentially urgent symptoms; diagnosis and treatment remain with qualified clinicians.

Sources for this section

Section reference: 2.5 Reason for Seeking Health Care — Nursing Skills 2e — Open RN; Wisconsin Technical Colleges System — https://wtcs.pressbooks.pub/nursingskills/chapter/2-5-reason-for-seeking-health-care/
Section reference: Basic Emergency Care: Approach to the Acutely Ill and Injured — World Health Organization and International Committee of the Red Cross — https://www.who.int/publications-detail-redirect/9789241513081

3. Establish a neurologic coordination baseline and notice change

PRC Annex A places perception and coordination disorders within Nursing Practice IV. A bounded baseline compares the person's usual function with current alertness and responsiveness, speech, pupils, motor strength, sensation, balance, gait, and ability to coordinate movement. Ask when a change began, compare sides or prior findings when appropriate, and document observations and timing. An unexpected finding calls for verification against history and baseline; it does not by itself identify a cause. A new gait or balance problem may have neurologic, sensory, musculoskeletal, or other contributors, so protect against falls, gather the relevant context, and communicate the change without assuming the cause. In older adults, medication use can also be relevant to dizziness or awareness changes; verify timing and context rather than assuming a drug caused the finding. Sudden loss of balance or coordination, vision loss, facial droop, weakness, or speech change can be signs of stroke. WHO describes suspected stroke as a medical emergency. For acute deterioration, ABCDE is a structured prompt to assess airway, breathing, circulation, disability/neurologic status, and exposure; the specific actions and authority depend on local scope and protocol. Assess immediate safety, note the onset and findings, and activate the current local response pathway; do not diagnose stroke, choose treatment, or delay for a routine check. Stable or chronic findings still need clear documentation and appropriate follow-up. Use facility-approved assessment tools and local policy; no single scale or threshold is prescribed here.

Sources for this section

Section reference: 26.2 Physical Assessment — Clinical Nursing Skills — OpenStax, Rice University — https://openstax.org/books/clinical-nursing-skills/pages/26-2-physical-assessment
Section reference: Stroke — World Health Organization — https://www.who.int/news-room/fact-sheets/detail/stroke
Section reference: The ABCDE Approach — Resuscitation Council UK — https://www.resus.org.uk/library/abcde-approach

Assess low intake and weakness

Question: A hospitalized person has eaten very little for several days and feels weak.
1. Clarify intake, symptoms, food preference, nausea, swallowing, access, and relevant history.
2. Assess hydration and weight trend and review ordered findings within role.
3. Communicate the pattern and collaborate on individualized support or referral.
Answer: Assess causes and risks first, then involve the care team.
Check: Do not force food, restrict fluids, or prescribe an unassessed diet.

Respond to sudden vision change

Question: A person reports sudden loss of vision in one eye with a severe headache.
1. Ask when it began and whether other neurologic or safety changes are present.
2. Assess the change and immediate safety within role without naming a diagnosis.
3. Activate urgent evaluation through the current local pathway and communicate the onset and associated symptoms.
Answer: Treat sudden focal sensory change as time-sensitive.
Check: A chronic vision history does not explain a new acute change.

Compare a new coordination change with baseline

Question: During a routine transfer, a client suddenly becomes less responsive, has new weakness, and cannot coordinate standing.
1. Stop the transfer and protect the client from a fall; do not ask the client to walk again to prove the change.
2. Assess immediate safety and ABCs within role, note the onset or last-known baseline, and describe observed neurologic changes.
3. Activate the current local urgent-response pathway and communicate the findings and timing; avoid assigning a diagnosis or selecting treatment.
Answer: Treat a sudden change as time-sensitive, stabilize immediate safety, document onset and findings, and escalate through the current pathway.
Check: Do not attribute the change to age, a chronic condition, or medication without assessment; the vignette does not authorize stroke diagnosis or treatment.

Common misconceptions

- Poor intake is not always a preference problem; symptoms, swallowing, disease, culture, and
- access matter.
- One weight or meal does not describe the whole nutrition trajectory.
- A familiar chronic sensory limitation does not make sudden loss benign.
- A warning pattern calls for assessment and escalation, not a diagnosis made from a vignette.
- An abnormal gait or coordination finding does not identify one cause; compare with baseline
- and assess relevant context.
- A sudden neurologic change is not made safe by a prior chronic deficit; record onset and use
- the current local escalation route without diagnosing.

Practice patterns

- For poor intake, collect a contextual history, assess swallowing and trends, then coordinate
- support.
- For sudden sensory loss, establish onset and associated changes and use urgent local
- assessment pathways.

Official scope references

Official scope: Enhanced Tables of Specifications for the Subjects in the Nurses Licensure Examination — Professional Regulation Commission; Professional Regulatory Board of Nursing — https://www.prc.gov.ph/sites/default/files/2025-10%20%20published%20Enhanced%20TOS%20nuse.pdf
Official scope: Nursing Licensure Examination Program, August 2026 — Professional Regulation Commission; Professional Regulatory Board of Nursing — https://www.prc.gov.ph/sites/default/files/exam%20progam%20aug%202026%20nle%20(nursing)%20signed.pdf
Official scope: Annex A: Enhanced Table of Specifications for Nursing Practice IV — Care of Clients with Problems in Nutrition, Gastro-Intestinal, Metabolism and Endocrine, Perception and Coordination (Acute and Chronic) — Professional Regulation Commission; Professional Regulatory Board of Nursing — https://www.prc.gov.ph/sites/default/files/2025-16%20annex%20A%20nurse.pdf

Instructional references

Instructional reference: 14.3 Applying the Nursing Process — Nursing Fundamentals 2e — Open RN; Wisconsin Technical Colleges System — https://wtcs.pressbooks.pub/nursingfundamentals/chapter/14-3-applying-the-nursing-process/
Instructional reference: Nursing Fundamentals — Open RN; Wisconsin Technical Colleges System — https://wtcs.pressbooks.pub/nursingfundamentals/

Instructional reference: 2.5 Reason for Seeking Health Care — Nursing Skills 2e — Open RN; Wisconsin Technical Colleges System — https://wtcs.pressbooks.pub/nursingskills/chapter/2-5-reason-for-seeking-health-care/
Instructional reference: Basic Emergency Care: Approach to the Acutely Ill and Injured — World Health Organization and International Committee of the Red Cross — https://www.who.int/publications-detail-redirect/9789241513081
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Instructional reference: Stroke — World Health Organization — https://www.who.int/news-room/fact-sheets/detail/stroke
Instructional reference: The ABCDE Approach — Resuscitation Council UK — https://www.resus.org.uk/library/abcde-approach
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